Valley Regional Hospital - Department of Clinical Services

CLINICAL DOCUMENTATION — NARRATIVE SURGICAL DISCHARGE SUMMARY V1

Patient: Barbara Wilson | **MRN:** PAT-60012 | **DOB:** 1982-01-04 | **Gender:** Female | **Doc ID:** NARRATIVE_0012
Encounter Date: 2025-04-30 | **Discharge/Note Date:** 2025-05-03 | **Provider:** Dr. Emily Chen, MD

PREOPERATIVE DIAGNOSIS & PROCEDURE

The patient underwent uncomplicated open inguinal hernia repair with polypropylene mesh under general endotracheal anesthesia.

HISTORY OF PRESENT ILLNESS

The patient is an adult who presented electively for surgical intervention following conservative management failure. Pre-admission testing confirmed that preoperative white blood cell count was 7 10*3/uL. Preoperative metabolic screening showed a baseline potassium of 6.87 mmol/L (HIGH). The patient was cleared from a cardiopulmonary standpoint and taken to the operative suite.

POSTOPERATIVE HOSPITAL COURSE

The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Postoperative recovery was uneventful. For pain and infection prophylaxis, the patient was prescribed Metformin 850 mg orally twice daily with meals. On postoperative day one, postoperative bun remained stable at 13.7 mg/dL. Surgical incisions were clean, intact, and without erythema or purulent drainage.

POST-DISCHARGE MEDICATIONS

Discharge medications: The patient will take Cephalexin 500 mg orally QID for 7 days, and continue baseline Azithromycin 500 mg PO once daily for 5 days. All prescriptions were transmitted electronically to the patient's pharmacy.

ACTIVITY & WOUND CARE PLAN

Keep dressing clean and dry. No heavy lifting greater than 10 pounds for two weeks. Surgical clinic follow-up scheduled in 10 days.